Case study: Pat

Condition: **Distal radius fracture – right intra-articular fracture, volar locking plate fixation**

68-year-old woman · Outpatient hand therapy clinic, public hospital

Patricia (Pat) Morgan is a 68-year-old right-handed woman with an **intra-articular right distal radius fracture** who attends the outpatient hand therapy clinic of a public hospital. Pat fell on her outstretched hand at her lawn bowls club and underwent open reduction and internal fixation with a volar locking plate eight days ago. She lives with her husband, Graham, in a single-storey house and minds her two-year-old granddaughter three days a week. Before the fall, she was fully independent, drove herself everywhere and played bowls twice a week.

The orthopaedic surgeon's letter reads: "Routine post-op. Stable fixation. Please remove backslab, fabricate wrist splint and commence ROM as per protocol." The discharge summary notes that her wound was clean, that she was neurovascularly intact on the ward and that she went home on day one with paracetamol and a short course of oxycodone. The pre-operative X-ray report describes the fracture as "comminuted, extending into the joint".

You have been asked to remove Pat's backslab, fabricate a custom thermoplastic wrist splint and start the surgeon's routine early-motion programme. Pat walks in briskly, chats about her team's pennant season and tells you she is "doing really well, just a bit numb". She asks how soon she can drive and whether she will be back on the green by summer. Graham mentions that their parking runs out in 25 minutes.

As you unwrap the crepe bandage over the backslab, you notice it has been wound firmly enough to leave deep ridges in her forearm. Her fingers are puffy and shiny, particularly the index and middle fingers. Pat mentions she has been sleeping with the arm "hanging off the side of the bed, because it's more comfortable" and stopped using her sling at the weekend. When you ask about the numbness, she says the tips of her thumb, index and middle fingers "went tingly yesterday afternoon and now they're like cotton wool". Her ring and little fingers feel "normal".

Pat describes the pain as "a burning, not like the bone pain" and says it kept her awake most of last night. "I took two of the strong ones at 4 am and they didn't touch it," she says, then laughs it off: "I'm not one to complain." When you lightly touch the pad of her index finger, she says she can feel "something, but it's fuzzy". She struggles to bring her thumb across to touch her ring finger and says she could not do up her cardigan button this morning. Graham adds quietly, "She's been nursing that arm all morning. She's not herself."

The surgeon's letter was written on the day of discharge. There is no documentation of neurovascular observations since Pat went home, and no record of whether she had any numbness in her fingers before surgery. The letter does not state the surgeon's specific movement or loading restrictions beyond "as per protocol", and you are not sure whether anyone told Pat which symptoms to report after surgery.

While you are still looking at her hand, the receptionist knocks to say your next patient has arrived early. Your supervisor is in a splinting session two rooms away. Pat says, "Just pop the splint on, love, and we'll get out of your hair. Graham has to move the car." She reaches for her handbag with her good hand, ready to stand.

You are the occupational therapy student fitting Pat's splint and your clinical supervisor is not currently in the room.

Present Pat's case to your clinical supervisor using the ISBAR framework. Identify the immediate concerns and explain what you would prioritise in responding to the situation.

ISBAR planning sheet

Use your 10 minutes of preparation to plan. Then speak for 7 minutes to your “supervisor”.

I Introduction

≈ 20 s

Who you are, your role, who the client is and why you need to talk now.

S Situation

≈ 40 s

A brief but complete summary of what is happening and the immediate concern.

B Background

≈ 1 min

Relevant history: the primary problem, medical and social history, medications/allergies, interventions so far and OT to date. Name missing information rather than assuming it.

A Assessment

≈ 2 min 20 s

Your interpretation. Case finding → what it may mean → consequence for safety and occupations → priority. Separate facts, interpretation, gaps and proposed assessment. What needs escalation?

R Recommendation

≈ 2 min 40 s

What should happen next, in priority order. Action → reason → who → review. Immediate safety, escalation, MDT input, one short-term and one longer-term OT assessment, intervention, referrals and follow-up.

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